FROM THE

EDITOR-IN-CHIEF

Humanity has known plenty of cruel seasons in the last century: wars, partitions, holocausts, famines—mass killers of all stripes. But nowhere did cruelty come packaged with such an acute strangeness of lived experience, and with such a world-conquering force, as during the Covid-19 pandemic. The Spanish Flu of 1918 was too distant in our generational memory. And the span of other, more recent zoonotic viruses like HIV, Ebola, SARS, avian flu or H1N1 stayed limited geographically or socially. No one remembered a time when the whole world had to down its shutters and go into hibernation. Quarantine, a word reserved for specific rare diseases, suddenly became a household phenomenon. Faces went behind masks—a veil of fear covered everyday life. Baleful new phrases like isolation and social distancing became the norm. If you didn't die of Covid-19, you could die of loneliness. In a world riven by wealth, class, religion and competing nationalisms, SARS-CoV-2 became the first great leveller of our times. Nobody was safe. For a while, it looked as if all those end-of-the-world disaster movies were coming true and the species itself was at risk.

That phase is behind us, by all reckoning. The referee in this game, the WHO, has not blown the whistle yet. That's because new variants, with a profusion of technical-sounding names like XBB, XBB1, BF.7, BA.5.1.7, BQ.1 and BQ.1.1, are causing a spike in infections in other countries. But a common theme binds them all. Mostly, they are variants of Omicron, the mildest version we have known. A temporary upper respiratory disorder, no oxygen dipping like during the second wave and, most importantly, no rush at the crematorium. In the face of growing immunity, the virus has chosen to focus on those mutations that help it to continue infecting. It has let go of those mutations that allowed it to reach cells deep in the lungs almost overnight. Coronaviruses in the past, too, have behaved similarly. The 1918 influenza virus, for example, was originally lethal enough to kill nearly 50 million people worldwide. But after two years, the virus shed its virulence and concentrated on immune escape mechanisms. Its descendants continue to infect, but are nowhere as deadly as the parent virus. Virologists believe Covid to be heading in the same direction.



March 30, 2020

That means, despite the WHO's cagey silence, we are most likely out of that black phase where a permanent pall of gloom hung over the world. The official global infection toll of 600 million will surely go up in ways that will not be quantified so obsessively. Does anyone count the number of humans catching a common cold? That is the status the virus seems to be aspiring to. Present? Very much. Lethal? Not very much. If current trends hold, the official global death toll—over 6.5 million—may rise, but only like it does with the flu, which is 300,000-600,000 a year. Barring some other unforeseen and macabre twist in the tale, the pandemic is well and truly over.

At least everyone is behaving like that—the government, the medical establishment, and the average person on the street, in the malls and restaurants, on the metro, or at work. Offices have largely relaxed the mask norm. Those temperature guns have disappeared even from airports. The fact that no one is checking means no one is any more worried. Whether the WHO pronounces this or not, the average person knows that the threat to life is considerably diminished, if not eliminated.

This week, we record this momentous, if unofficial cessation of the pandemic for our cover story. Senior Associate Editor Sonali Acharjee had tracked the progress of the virus, with all its mutations and the devastation it had caused after the first Covid-19 case was detected in India on January 27, 2020 in a 20-year-old woman who had returned to Kerala from Wuhan. Since then, the magazine has carried 33 cover stories on Covid-19, and several others. For this issue, Acharjee has mapped this great ebbing of the tide, where everyone from scientists to doctors to the officialdom and the general populace seems to believe there won't be a fourth wave. She discovered some heartening facts, some we would have found unbelievable even two months ago. Covid speciality hospitals have all been withdrawn from frontline duty and turned back to regular business. Excellent stocks of Covid vaccine are lying unused. As on October 16, only 23 per cent of the eligible had taken their third precautionary dose, 45 per cent among those above 60. "People think Covid is over, so they aren't coming in for the shot. We are trying through reminders and awareness building to improve the number," says Dr Suresh Kumar, medical director, LNJP, Delhi. But even the government presently has no plans to introduce a fourth shot.

Even if some of the dismantlings seems incredible, there are also permanent gains for India's health infrastructure. "One day, we will look back and be able to see the benefits After Covid (AC) and Before Covid (BC)," says Dr Devi Shetty, chairman, Narayana Health. From just one lab for testing, we today have over 4,000. Also, over 56 labs are now geared to do genomic sequencing. The Centre has funded 1,222 PSA plants through PM Cares, and they can produce 1,750 metric tonnes of captive oxygen daily. Much of this additional capacity is expected to be diverted to other diseases. Around 12,000 vaccination sites could give out 10 million doses a day at the programme's peak. Despite severe faltering, the government ultimately did stupendous work in vaccinating 0.95 billion Indians, or nearly 73 per cent of our population, with double doses and a total of 2.19 billion shots in a matter of 21 months.

Based on the universal scientific and governmental response, let us assume the pandemic is over, while Covid lingers on as just another flu. Despite the 30 months of despair we all went through, there were some beneficial lessons that we learnt from this nightmare. Indian health infrastructure is woefully inadequate. We have 0.5 beds for 1,000 people; at district hospitals, that dips to 0.24 beds. The WHO mandates 5:1,000. There is a 76 per cent shortage of specialist doctors in rural India. But the pandemic also showed us what the country could achieve overnight—handling almost 400,000 new cases daily at the peak of the Delta wave. It is essential to sustain the momentum and continue adding much-needed beds, doctors, intensive care units and hospitals so that we are prepared not just for Covid but any disease. The plight of the urban poor was brought home to us when millions migrated back to their villages during the pandemic. They need a safety net. We also learnt the importance of self-reliance—Aatmanirbhar Bharat, which the prime minister has been propagating vigorously, needs to be accelerated.

Right now, India is off to celebrate its first Diwali without masks since 2019. There can be no better omen than this on our most auspicious day. Here's wishing you a joyful Diwali.



(Aroon Purie)